

Pennsylvania's Individual Health Insurance Market at the Crossroads: A Comprehensive Policy Analysis of the 2026 Premium Shock, Attrition Mechanics, and Regulatory Challenges



CRISIS ACTION SIDEBAR: IF YOU RECEIVE A SURPRISE OR SHOCK BILL

1. **Do Not Pay It Immediately:** Request a fully itemized bill from the provider and an Explanation of Benefits (EOB) from your health insurance carrier.
2. **Check for Directory Errors:** If you selected this provider based on an inaccurate carrier directory, your out-of-pocket costs are legally capped at your plan's in-network cost-sharing amounts under **28 Pa. Code § 9.725**.
3. **Verify Notice Timelines:** Under **Act 252 of 2023**, out-of-network providers must give you written notice at least 7 days in advance of rendering non-emergency services, or the balance bill is presumed illegal.
4. **File an Official Complaint:** If the carrier or provider refuses to adjust the bill, file an immediate complaint with the **Pennsylvania Insurance Department (PID)** within 30 days at pa.gov/consumer or call **1-866-722-6675**.

Section 1: Executive Crisis Overview and Macroeconomic Drivers

The individual health insurance market in Pennsylvania has entered a period of unprecedented structural volatility. The expiration of the federal Enhanced Premium Tax Credits (EPTCs) on December 31, 2025, combined with a Pennsylvania Insurance Department (PID)-approved average individual-market rate increase of 21.5% for the 2026 plan year, has triggered an average net premium spike of 102% for subsidized enrollees. Concurrently, the commonwealth's primary statutory mechanism for state-level premium relief—the Act 54 State Health Insurance Exchange Affordability Program—remains entirely unfunded. This is due to a protracted legislative budget impasse for the fiscal year 2026–27 that has missed its June 30, 2026 deadline, marking the fifth consecutive year of budget delays.

These dual cost shocks have completely altered the affordability landscape. During the enhanced subsidy era from 2021 through 2025, federal funding insulated enrollees from the true retail cost of health coverage, capping monthly premiums at a maximum of 8.5% of household income and allowing Pennie enrollment to peak at a record 497,000 enrollees. The sudden reversion to pre-2021 subsidy structures has stripped approximately \$600 million in annual premium tax credits from the Pennsylvania market, forcing thousands of families to choose

between maintaining coverage and paying for basic necessities like housing, food, and utilities.

Section 2: Chronological Reconciliation of Enrollment Attrition

A rigorous assessment of Pennsylvania's individual market requires resolving the apparent contradictions in reported enrollment drops. While initial reports from the close of the 2026 Open Enrollment Period (OEP) on January 31, 2026, cited approximately 85,000 terminations, subsequent regulatory updates published cumulative cancellation figures of 120,000 through early April and over 160,000 by June 1, leaving the state's effectuated enrollment at 443,024. Rather than representing contradictory datasets, these figures trace a sequential chronological progression of market exit behaviors driven by distinct financial and regulatory triggers.

Measurement Point	Reporting Date	Attrition Volume	Primary Driver of Market Exit	Likely Overlap and Data Integration	Source
Open Enrollment Close	January 31, 2026	85,000	Active non-renewal and immediate plan abandonment.	Distinct group of consumers who dropped coverage during the OEP rather than pay the first 2026 premium.	
Mid-Spring Reconciliation	April 6, 2026	~120,000	Expiration of preliminary grace periods for non-subsidized buyers.	Combines the original 85,000 non-renewals with early post-OEP dropouts.	
Initial Grace Period Close	May 1, 2026	145,000	Expiration of the mandatory 90-day premium payment grace period.	Incorporates enrollees receiving tax credits who failed to pay their portion of Feb/Mar/Apr premiums.	
Late-Spring Update	June 1, 2026	160,000+	Cumulative attrition including mid-year nonpayment exits and unsubsidized drops.	Complete cumulative picture of all plan year exits, offset by limited special enrollment period additions.	

At the close of the OEP on January 31, 2026, the 85,000 terminations represented gross non-renewals. These were consumers who actively logged into Pennie, compared their 2025 subsidized rates against the unsubsidized 2026 premiums, and immediately chose to drop coverage rather than complete a single premium payment for the new plan year. The escalation to 120,000 by early April and 145,000 by May 1 represents the operationalization of federal grace periods. Under Affordable Care Act regulations established in 45 C.F.R. § 156.270(d), enrollees receiving Advance Premium Tax Credits (APTCs) are entitled to a mandatory three-month grace period to resolve outstanding premiums before an insurer can legally terminate their coverage. For individuals who selected a plan during the OEP but failed to pay their portion of the January or February premiums, their coverage remained conditionally active. Insurers did not execute final terminations for these accounts until late April, causing a delayed spike in reported cancellations.

By June 1, 2026, cumulative cancellations exceeded 160,000, leaving Pennsylvania's total effectuated individual market enrollment at 443,024. This final wave of attrition reflects mid-year dropouts among unsubsidized consumers who attempted to maintain coverage but eventually defaulted as the absolute premium burden conflicted with other essential household expenses.

Section 3: Regional Market Dynamics (Rating Areas 1–9)

The impact of the 2026 premium shock is not uniform across the commonwealth's nine rating areas. While the statewide average rate increase is 21.5%, geographic disparities in carrier density, local provider disputes, and regional demographics have created distinct risk profiles.

PENNSYLVANIA GEOGRAPHIC RATING AREAS		
(Erie Corridor)	(Rural Tier)	(Poconos/Scranton)
(Altoona/Johnstown)	(Allentown/State Coll)	(Harrisburg Hub)
(Pittsburgh Metro)	(York/Lancaster)	(Five Counties)

Rating Area 3: Northeast / Poconos

Rating Area 3 (Scranton/Wilkes-Barre and the Poconos) exhibits the widest carrier rate divergence in the state. Oscar Health Plan of Pennsylvania was approved for a 23.1% rate increase, while Partners Insurance Company (operating as Jefferson Health Plans) was approved for a 10.1% rate decrease—the only rate reduction in the entire Pennsylvania individual market. This extreme variance makes active consumer shopping critical; enrollees who remain on auto-renewal are exposed to significant unnecessary financial penalties.

Rating Area 6: Central Pennsylvania / Lehigh Valley

Rating Area 6 is experiencing severe network instability due to an ongoing commercial contract dispute between UnitedHealthcare (UHC) and Lehigh Valley Health Network (LVHN). UHC Medicare Advantage members lost in-network access to LVHN on January 26, 2026, while commercial and employer-sponsored plan members lost access on April 26, 2026. This contract termination affects approximately 50,000 commercial enrollees, including veterans utilizing the VA Community Care Network.

Rating Area 8: Philadelphia Metro

As the most carrier-dense rating area in the commonwealth, Philadelphia enrollees have access to Keystone Health Plan East (Independence Blue Cross), QCC, Oscar, Ambetter, Partners, and Health Partners Plans. However, Independence Blue Cross is actively phasing out broad PPO plans in favor of tiered "Proactive" HMO networks. While this keeps baseline premiums lower, it exposes consumers to high cost-sharing if they seek care outside the plan's preferred provider tiers.

Rating Areas 9 and 2: Capital Region and North Central Rural Tier

Rating Area 9 (Harrisburg) and Rating Area 2 (Coudersport/Rural Tier) represent highly vulnerable, carrier-thin markets. Capital Advantage Assurance Company (a subsidiary of Capital Blue Cross) operates heavily here and was approved for a 24.56% rate increase. While Pennsylvania's Section 1332 reinsurance program offsets approximately 4% of baseline premiums, this buffer is entirely insufficient to offset the double-digit rate hike and the loss of federal tax credits.

Section 4: The Economic Realities of the Subsidy Cliff and Silver CSR Mechanics

The loss of federal enhanced subsidies has reinstated the federal subsidy cliff at 400% of the Federal Poverty Level (FPL). Because exchange calculations for the 2026 plan year utilize the prior year's poverty guidelines, the 2025 FPL thresholds serve as the definitive baseline for 2026 premium assistance and eligibility. Under these guidelines, the 100% FPL baseline is established at \$15,650 for a single individual and \$21,150 for a family of two. Consequently, the 400% FPL subsidy cliff is set at \$62,600 for single applicants and \$84,600 for two-person households.

The mathematical formula used to calculate a household's FPL percentage is:

$$\text{FPL}\% = \left(\frac{\text{Annual Gross Household Income}}{\text{Baseline FPL for Household Size}} \right) \times 100$$

For a single median-income full-time worker in Pennsylvania earning \$62,529, their FPL ratio is calculated as:

$$\text{FPL}\% = \left(\frac{\$62,529}{\$15,650} \right) \times 100 \approx 399.55\%$$

This worker sits at exactly 399.55% of the FPL—literally one paycheck away from crossing the 400% threshold where all federal premium assistance disappears.

For couples, crossing the 400% threshold (\$84,600) completely eliminates federal premium assistance. In its official rate announcement, the PID highlights a 60-year-old York County

couple earning \$82,000 whose annual premium skyrocketed from \$7,032 to \$35,712—a 408% increase.

However, a strict FPL calculation reveals an analytical discrepancy in this example:

$\text{FPL Ratio} = \left(\frac{\$82,000}{\$21,150} \right) \times 100 \approx 387.71\%$

At 387.71% of the FPL, this couple is technically below the 400% cliff and should remain eligible for sliding-scale subsidies under standard pre-2021 rules. The skyrocketing \$35,712 premium cited by the PID represents the *gross unsubsidized cost* of the plan, illustrating the worst-case scenario if the couple's income slightly increases or if they are forced to buy off-exchange. This highlights the critical importance of consumers obtaining direct determinations from Pennie rather than relying on generalized models.

Household Income (\$)	% of 2025 FPL	EPTC Net Monthly Premium (2025)	Post-EPTC Net Monthly Premium (2026)	Absolute Annual Premium Cost (\$)	Net Premium as % of Income
\$42,300	200.0%	\$70	\$150	\$1,800	4.3%
\$63,450	300.0%	\$210	\$480	\$5,760	9.1%
\$82,000	387.7%	\$580	\$1,930	\$23,160	28.2%
\$84,600	400.0%	\$586	\$2,827	\$33,924	40.1%
\$95,175*	450.0%	\$674	\$2,827	\$33,924	35.6%
\$105,750*	500.0%	\$749	\$2,827	\$33,924	32.1%

**Note: Extrapolations for the 450% and 500% FPL rows assume constant benchmark plan pricing. While the percentage of income spent on premiums appears to decline in these rows, this occurs because the consumer is paying the flat, unsubsidized retail cost of the plan. The absolute dollar amount paid remains at the maximum retail price, which itself rose 21.5% statewide per PID filings, meaning the dollar burden continues climbing year-over-year.*

This premium shock has altered consumer selection patterns. Driven by affordability constraints, Silver-tier enrollment on Pennie declined as consumers migrated to Bronze-tier options to escape premium hikes. Bronze-tier enrollment surged by roughly 30% (+33,000 enrollees) statewide. While Bronze plans feature lower monthly premiums, they expose families to significant financial risk due to structurally higher cost-sharing. For 2026, the average Bronze-tier deductible is \$7,186, compared to \$5,304 for standard Silver-tier plans. Furthermore, Cost-Sharing Reductions (CSRs), which lower out-of-pocket maximums, deductibles, and copayments, are legally restricted to Silver-tier plans. CSR adjustments are entirely unavailable to enrollees who migrate to Bronze or Gold tiers, meaning lower-income households shifting to Bronze plans to escape premium hikes are forfeiting vital federal cost-sharing support.

This migration is particularly acute given the billing complexities surrounding the transition from preventive to diagnostic care. Under the Affordable Care Act, routine preventive screenings are covered at 100% with \$0 cost-sharing. However, if a clinician identifies an active medical condition during a routine screening, the service transitions from preventive to diagnostic. To document this transition, providers append *modifier 25* to the Evaluation and Management (E&M) billing code, indicating that a significant, separately identifiable evaluation was performed on the same day. For an enrollee in a Gold or Silver plan, this billing modification triggers standard copays or coinsurance. For a consumer in a newly selected Bronze plan with a \$7,186 deductible, the transition from a \$0 preventive visit to a fully deductible-applicable diagnostic visit frequently results in unexpected, multi-hundred-dollar out-of-pocket charges.

Section 5: Quantitative Audit of Provider Network Stability and Directory Inaccuracies

The stability of Pennsylvania's individual health insurance market is further undermined by systemic discrepancies in carrier-reported provider networks. To quantify this issue, the PID commissioned comprehensive, large-scale secret shopper audits to evaluate the physical and behavioral health networks of carriers operating in the ACA marketplace.

Physical Health Network Survey

The PID Physical Health Survey, fielded from March 20 to August 18, 2023, evaluated all eight participating carriers across seven adult specialties and general pediatrics. The audit revealed deep structural barriers to consumer access:

- **Inaccuracy Rates:** Almost 50% of all secret shopper calls encountered directory errors, including inactive phone numbers, incorrect specialty classifications, or inaccurate network participation statuses.
- **Appointment Failures:** 20% of simulated patients could not secure an appointment with any provider, even after making ten unique calls to listed in-network clinicians.
- **Wait Times:** For successful appointments, the average wait time was 60.3 days (median of 41 days). Specialized care wait times peaked at 91.8 days for endocrinology.
- **Capacity Bottlenecks:** 15% of all calls encountered capacity limits, where listed providers refused to accept new patients or were not scheduling new appointments.

Behavioral Health Network Survey

The PID Behavioral Health Survey, fielded from August 16, 2023, to February 27, 2024, evaluated 8,306 randomly selected outpatient mental health counselors from regulatory filings across individual market networks. This audit revealed even more severe network deficiencies:

- **Filing-to-Directory Gaps:** 20% of counselors submitted in official carrier regulatory filings could not be located in consumer-facing online directories. Only 44.0% of located providers had a complete, identical match for all contact information between regulatory filings and the public directory.
- **Inaccuracy Rates:** Of the 2,152 directory listings successfully contacted, 76.0% contained at least one inaccuracy. Phone number errors were the primary driver, affecting 56.6% of verified listings.
- **Appointment Failures:** Secret shoppers secured appointments in only 14.9% of verified attempts.
- **Refusal Rates:** 9.1% of contacted clinicians explicitly stated they were not offering appointments to new patients, despite being listed as active and accepting patients in carrier directories.

Longitudinal Re-Audit of Inaccuracy Persistence

To assess whether insurers correct these errors, a longitudinal follow-up study re-contacted 5,170 providers with identified inaccuracies after an interval of 117 to 280 days. Despite federal mandates under the No Surprises Act requiring insurers to verify and update directories every

90 days, the audit demonstrated persistent compliance failures :

- **Persistent Errors:** 44.8% of provider listings continued to exhibit at least one directory inaccuracy.
- **Unreachable Providers:** Shoppers were entirely unable to reach 24.6% of the listed providers.
- **Correction Deficit:** Only 11.6% of inaccurate listings had been corrected to full accuracy, and only 19.0% of inaccurate listings had been removed from the directory. The mean duration of persistent directory inaccuracies was 190 days.

| Performance Indicator | Physical Health Network | Outpatient Behavioral Health | Longitudinal Re-Audit (Persistence) | | :--- | :--- | :--- | :--- | | **Directory Inaccuracy Rate** | ~50.0% of unique calls | 76.0% of verified listings | 44.8% of listings remain inaccurate | | **Failed**

Contact/Unreachable | Not explicitly segmented | 67.7% of attempts (4,505 providers) | 24.6% of listings remain unreachable | | **Appointment Success Rate** | 21.1% to 24.3% per unique call | 14.9% of verified listings | 11.6% corrected to full accuracy | | **Mean Wait to Appointment** | 60.3 days (Median: 41 days) | 33.2 days (Median: 27 days) | Mean of 190 days between reviews | | **New Patient Refusal Rate** | 15.0% (capacity bottlenecks) | 9.1% of active listings | 19.0% of inaccurate listings removed |

These *ghost networks* have significant legal implications. If an enrollee relies on inaccurate directory information and is treated by an out-of-network provider, Pennsylvania consumer protections intervene. Under 28 Pa. Code § 9.725, the consumer's financial liability is strictly capped at applicable in-network cost-sharing amounts.

Additionally, Act 252 of 2023 establishes a legal presumption that any undisclosed out-of-network balance bill resulting from a directory error is illegal, shifting the dispute resolution burden from the patient to the insurer. Providers must give written notice at least 7 days in advance of rendering non-emergency out-of-network services to bypass this presumption.

These protections differ from the federal Independent Dispute Resolution (IDR) process under 45 C.F.R. § 149.510, which operates exclusively as a provider-versus-plan arbitration mechanism; individual consumers cannot access or file an IDR claim.

Section 6: Pharmaceutical Cost Pressures: GLP-1 Weight-Loss Pricing and Risk Pool Morbidity

The financial volatility of Pennsylvania's individual market is increasingly driven by pharmaceutical spend, specifically the rapid adoption of glucagon-like peptide-1 (GLP-1) receptor agonists. In their 2026 rate filings, major carriers, including Capital Advantage Assurance Company and Health Partners Plans, cited medical and prescription drug cost trends as primary drivers for rate changes, resulting in approved individual-market increases of 24.56% and 16.70% respectively.

This premium pressure is closely tied to the shifting landscape of GLP-1 pricing and coverage. Under pricing agreements finalized between the Trump administration and manufacturers Eli Lilly and Novo Nordisk in November 2025, brand-name GLP-1 weight-management injectables are available directly to self-paying consumers via the TrumpRx platform for a targeted average price of approximately \$350 per month.

These shifts have transformed commercial coverage, creating a direct spillover effect into the individual market. In 2025, GLP-1 medications for weight loss accounted for over 10% of all annual prescription drug claims among employer-provided health plans in the United States.

Sixty-six percent of large employers reported that covering GLP-1s had a significant impact on their total pharmacy spend, prompting a wave of coverage terminations. For the 2026 plan year, major commercial insurers, such as Harvard Pilgrim Health Care and Blue Cross Blue Shield of Michigan, dropped weight-loss GLP-1 coverage entirely.

When large employers terminate GLP-1 benefits, employees are forced to choose between paying full out-of-pocket prices, utilizing the \$350 monthly TrumpRx cash-pay platform, or migrating to the individual ACA marketplace in search of plans that maintain favorable drug formularies. This migration shifts individuals with high-cost chronic conditions into Pennsylvania's individual exchange, inflating carrier medical loss ratios and driving up premiums for all individual plan holders.

Program / Platform	Intended Target Population	Out-of-Pocket Cost (\$/Month)	Effective Date	Coverage Exclusions / Criteria	Source
Commercial Retail	Unsubsidized cash-paying patients.	\$1,349.02	Immediate	No restrictions; full retail list price.	
TrumpRx DTC	Self-paying direct buyers (vials).	~\$350.00	Early 2026	Bypasses traditional pre-authorization requirements.	
Medicare Part D Pilot	Government-insured beneficiaries.	\$50.00	Mid-2026	BMI of at least 30, or at least 27 with comorbidity.	
Novo Nordisk Price Cut	Commercial and exchange enrollees.	\$675.00	January 1, 2027	Unified list price for Ozempic/Wegovy/Rybelsus.	

Section 7: Modeling Legislative Pathways and Premium Relief Timelines

The future stability of Pennsylvania's individual health insurance market depends on the resolution of parallel state and federal legislative debates. At the federal level, the legislative debate centers on the extension of the Enhanced Premium Tax Credits. On January 8, 2026, the U.S. House of Representatives passed a three-year clean extension of the EPTCs by a bipartisan vote of 230 to 196, carrying the subsidies through December 31, 2028.

However, this clean extension faces gridlock in the Senate. A bipartisan coalition of senators is negotiating a compromise alternative—the Consumer Affordability and Responsibility Enhancement (CARE) Act—which would extend the EPTCs for two years but introduce major structural modifications, including mandatory income verification, minimum monthly premium payments on enrollees, and restructured cost-sharing reductions.

At the state level, the principal vehicle for premium mitigation is the State Health Insurance Exchange Affordability Program, established under Act 54 of 2024. If funded, the program would redirect a portion of the state's reinsurance funds to provide premium subsidies for households earning between 151% and 300% of the FPL.

A state investment of \$50 million (roughly one-twelfth of the \$600 million in lost federal EPTC

funding) is projected to produce immediate market improvements, reducing average premiums by 9% to 12% and recouping approximately 25% of the individuals who dropped coverage during the 2026 premium shock, restoring over 40,000 enrollees to the Pennie exchange. Despite being operationally ready to deploy, the program remains entirely unfunded because the General Assembly budget delay for FY2026–27 has now surpassed last year's delay.

Policy Pathway Scenario	Projected Resolution	Impact on 2027 Net Premiums	Impact on Pennie Enrollment	Forward-Looking Confidence	Source
Scenario A: Clean Federal Extension	Mid-to-Late 2026	Net premiums immediately drop back to 2025 levels, reducing out-of-pocket costs by over 50%.	Captures majority of the 160,000 lost enrollees; enrollment returns toward 500,000.	Moderate	
Scenario B: Senate CARE Act Compromise	Fall 2026	Net premiums stabilize, but lower-income enrollees face new mandatory monthly payments.	Restricts coverage drops among middle-income enrollees, but triggers secondary attrition of 5%–8%.	High	
Scenario C: Persistent Legislative Inaction	Ongoing Gridlock	Baseline premiums for the 2027 plan year face another round of double-digit increases.	Attrition continues; cumulative cancellations projected to exceed 200,000 by Dec 2026.	High	

Scenario A: Clean Federal Extension (Status Quo Ante)

The Senate passes the House-approved three-year clean extension of the EPTCs. Net premiums for subsidized enrollees immediately drop back to 2025 levels, reducing average out-of-pocket costs by over 50%. Baseline premiums decline by an estimated 3% to 5% as healthier enrollees re-enter the risk pool. Pennie enrollment recovers, recapturing the majority of the 160,000 lost enrollees and pushing effectuated enrollment back toward the half-million mark. This scenario relieves legislative pressure on the General Assembly to fund the Act 54 state subsidies.

Scenario B: Senate CARE Act Compromise

The Senate passes a two-year extension of the EPTCs with income limits, minimum premiums, and CSR structural changes. Net premiums stabilize, but lower-income enrollees (100%–150%

FPL) face new mandatory monthly premiums, and modified CSR funding increases out-of-pocket expenses. This scenario restricts coverage drops among middle-income enrollees (250%–400% FPL), but administrative hurdles (such as the elimination of conditional eligibility) trigger secondary attrition of approximately 5% to 8% among vulnerable populations and lawfully present immigrants.

Scenario C: Persistent Federal and State Inaction

Congress fails to pass any subsidy extension, and the Pennsylvania state budget gridlock continues, leaving Act 54 unfunded through the fall of 2026. Baseline premiums for the 2027 plan year face another round of double-digit increases, driven by worsening risk-pool morbidity. Net premiums remain at their elevated 2026 levels. Attrition on Pennie continues. Cumulative cancellations are projected to exceed 200,000 by December 2026, driving total effectuated enrollment below 400,000. Uncompensated care costs at rural and safety-net hospitals rise sharply, increasing state Medicaid obligations and straining the commonwealth's broader healthcare economy.

Disclaimer

Specific subsidy calculations, carrier decisions, or coverage determinations must be obtained directly from official sources (Pennie, the Pennsylvania Insurance Department, or your insurance carrier) before making final financial or healthcare decisions.

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